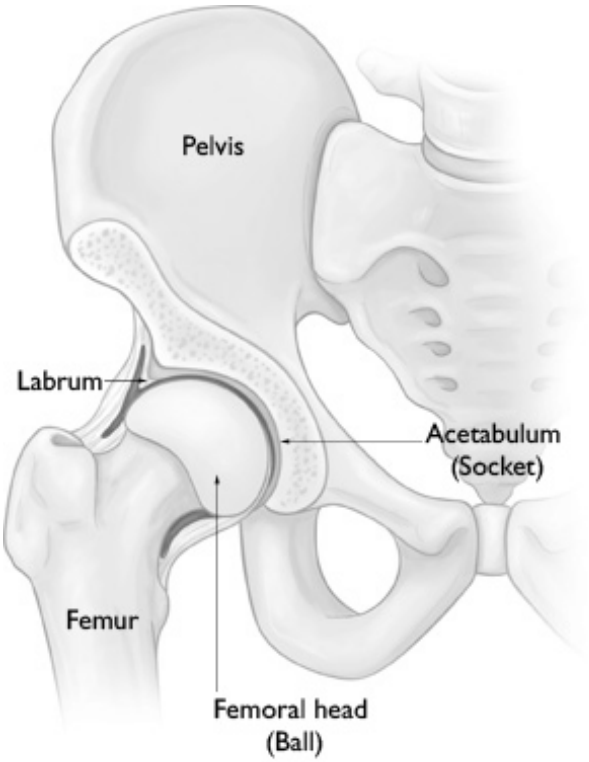
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# What to Expect from your Hip Arthroscopy Surgery




*Hip Joint Diagram – Front View*

# A Patient's Guide

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I think many people can find going to see the doctor an emotional and sometimes difficult experience. I think it is more stressful when potential surgery is a consideration. While many of the things we discussed are simple, one may quickly forget the details. Hopefully this letter will serve to remind you of our meeting, review what was discussed, and answer any questions that arise.

***Indications for Hip Arthroscopy***

Since the early 20th century, when hip arthroscopy was regarded as being almost impossible to undertake, the procedure has developed in leaps and bounds. Presently there are many reasons why hip arthroscopy may be recommended. Perhaps the two most common current indications for hip arthroscopy include the presence of symptomatic FAI or an acetabular labral tear, or both.

***Femoroacetabular Impingement (FAI)***

FAI (or hip impingement) is a condition affecting the hip joint and is characterized by abnormal contact between the femoral head (hip ball) and the rim of the acetabulum (hip socket) leading to damage to the articular cartilage (lining) in the acetabulum, or to the labrum of the hip, or both. The labrum is a ring of cartilage that surrounds the acetabulum. Damage to the labrum and/or articular cartilage will likely cause pain. An abnormality in the shape of the femoral head or acetabulum, or both, may cause FAI. Activities that involve recurrent hip motion can increase the frequency of this abnormal contact.

FAI can affect all age groups from the early teens to throughout adult life, and is being increasingly recognized as one of the predisposing factors for osteoarthritis of the hip. While hip arthroscopy may reduce the chance of developing osteoarthritis, it does not eliminate it. Hip arthroscopy can be used to reshape the socket and/or the femoral head to prevent impingement, and aims to decrease the risk of developing osteoarthritis, as well relieving current painful symptoms.

***Hip Dysplasia***

Hip dysplasia, or a shallow acetabulum (socket), can increase the stress on the acetabular cartilage and labrum with regular or increased activities. With a shallow socket, some of the force that would normally be placed on the socket is transferred to the labrum, which can lead to a labral tear. In cases of mild or borderline dysplasia, repairing the labrum can relieve the painful symptoms within the hip with arthroscopy. However, it does not deepen the socket. A shallow socket can increase the risk of developing osteoarthritis in the hip. The goal of a hip arthroscopy would be relieving the current painful symptoms in the hip. It will not eliminate the risk of developing arthritis. In cases of advanced dysplasia, larger reconstructive surgeries (like a periacetabular osteotomy or PAO) is an option to increase the coverage.

Hip dysplasia and hip impingement are opposite ends of a spectrum (over-coverage versus under-coverage). Please check with me if you are unsure about your underlying diagnosis.

***Acetabular Labral Tears***
The labrum, which surrounds the acetabulum, can be partially damaged or torn. This is usually associated with FAI, but not always so. It can also occur with hip dysplasia (a shallow hip socket), after an injury or trauma, or even from over-use. With hip arthroscopy, the labrum can be either repaired, or in some cases debrided (remove the damaged tissue only). Occasionally a labrum can also be reconstructed with a graft. MRI and/or CT scans usually, but not always, reveal a labral tear.

***What I Recommend***
In view of your description of your problem, my physical examination and review of the X-Rays/MRI, I believe that surgery is medically indicated on your hip. The indications for surgery are persistent pain interfering with your activities of daily living, work and/or sports that has not resolved with conservative treatment.

***Alternative Treatment Options***
While I believe surgery is the best course of action, other treatments are available. These vary from no treatment, just living with the condition, to physical therapy exercises, medication or injections. You may have tried some or all of these already. We can arrange for any or all of these options if you desire.

***Stem Cell Treatments***
Stem cells are used to repair diseased or damaged tissues. While stem cell, connective tissue matrix, or PRP injections may not heal your hip on their own, there is emerging evidence that it can be used to augment and strengthen your surgical repair.

These treatments can be done during your surgery. Unfortunately, they are currently not covered by insurance. Please contact us if you would like more information or would like to include this with your surgery.

***How We Perform Hip Arthroscopy***
The bones of the hip joint (the ball and socket) are separated by approximately 1cm by applying traction to the foot while wearing a special boot. By distracting the hip, this provides enough room for a small telescope ('arthroscope') to be introduced into the joint. Initially, air and/or fluid are injected into the hip, under x-ray guidance. Once correct placement of the instrument has been confirmed, two, three, or sometimes four small incisions (portal sites) are made on the side of the hip. Each of these incisions generally measures approximately 5-10 mm in length.

Through these small portals, the arthroscope and instruments are passed into the joint. We will then be able to visualise the hip joint, identify the problem(s), and proceed appropriately. The operation duration will vary depending on the problem in the hip joint but can last from 40 minutes to 60 minutes, or even more. During the surgery, further x-rays may be taken, for example, to confirm adequate removal of bone.

At the end of the procedure, medications may be injected into the hip to minimize pain after the surgery. The small incisions are closed with sutures. Finally, a further dressing is placed over the incisions.

When you arrive at Texas Orthopedic Hospital a nurse will have you sign the Operative Permit. The name of the operation I will be doing is "Hip Arthroscopy: Labral Repair, Osteoplasty". Additional procedures may be included depending upon the problem(s) in or around your hip joint.

***Anesthesia***

The surgery is performed under general anesthesia. You will meet the anesthesia team during your pre-op appointment, and again the day of surgery. If you are susceptible to nausea or vomiting after anesthesia, or have a history of complications with anesthesia, please let our anesthesia team know.

***Time in the Hospital***

Patients enter the hospital in the morning, have the surgery and go home the same day. This is called outpatient surgery.

***What do I need to do prior to surgery?***

There is a pre-registration and a pre-admission process. Both must be completed no less than 7-days prior to your surgery. Both processes can be completed when you come in to pre-admission; however, the pre-registration process may also be completed on-line.

Pre-Registration

To pre-register on-line please visit www.texasorthopedic.com and go to the Menu tab in the top left corner. Scroll down to the "Patients & Visitors" tab, and click on "Online Pre-Registration". In the middle of the page under "MyHealthOne", click on "Create your account or Sign in". You will then be taken though the pre-registration process.

Pre-Admission Testing (PreAdmit)

***The PreAdmit appointment is MANDATORY*** and is necessary in order to obtain surgical clearance for you to receive anesthesia for your surgery. While in the PreAdmit department on the 3rd floor of Texas Orthopedic Hospital, you will be interviewed by the nursing and anesthesiology teams. If you would like to schedule an appointment, please call **713-794-3525** between the hours of 8 a.m. and 5 p.m. If you would like to be seen on a walk-in basis, their office hours are 8 a.m. to 5 p. m., Monday through Friday. This needs to be completed within 30 days but AT LEAST ONE WEEK PRIOR to surgery.

***Using Ice or Heat After Surgery***

Ice is recommended instead of heat. An ice pack should be applied to the hip (not directly on the skin) for at least 20 minutes, three times a day.

The use of a cold therapy device (e.g. PolarCare) device helps decrease pain and swelling after surgery. The use of the device may or may not be covered by the particular health insurance plan you have selected. You should check with your insurance company. It is optional to use. A cold-therapy device can also be purchased for you to own. Please contact us if you have questions about this.

***After Hip Arthroscopy***

Usually, you will feel some discomfort in your hip. In addition, the discomfort can be experienced in the lower back, buttock, knee and ankle. The discomfort can normally be reduced with appropriate pain medication. There is often some swelling and bruising in the groin, buttock and thigh. This is caused by the fluid used during the

surgery. The swelling reduces over the following few days.

Depending on the procedures that were performed, you may be asked to limit the amount of weight you put through your operated leg. Consequently, you may require crutches for a few days, or weeks depending on what specific surgery has been performed. The specific amount of time with the crutches will be outlined in your protocol.

You will also use a brace for 4 weeks when walking that limits hip motion. It may be removed when bathing, exercising, sitting, and sleeping.

You will return to the clinic 10 to 14 days after the surgery for your first post-operative visit typically with our Physician's Assistant. At this appointment, your incisions may be inspected. The incisions are closed with buried sutures under the skin, so no sutures will need to be removed. A further explanation of the surgery will be provided and there will be an opportunity for specific queries to be answered. Any subsequent appointments will be arranged and will be guided by the surgery performed.

We will employ an appropriate rehabilitation program for you following the surgery, depending on the surgery performed. Your physical therapist will guide your return to sporting activities (running etc.) depending on your progress. This is extremely variable between individuals, depending on the surgical findings and the length of symptoms prior to surgery.

My personal feeling is that the goal of this operation is to eliminate or diminish pain and allow you to comfortably perform normal activities of daily living.

In the majority of cases, 6 to 8 weeks after surgery you should be walking relatively pain-free. Remember, however, that it may take 6 to 9 months (or more) to fully recovery and to return to an elite level of competition/fitness. Any unexpected increase in pain can be treated with ice packs and anti-inflammatory medication.

There are some activities to avoid or take care with up to 8 weeks following surgery. These include the following:

- Prolonged standing, especially on hard surfaces.
- Prolonged walking i.e.; around shopping centres.
- Heavy lifting
- Squatting / crouching
- Sleeping on your side. Try to sleep on your back. If you must sleep on your side, sleep on the unoperated side, with a pillow under your operated leg – to hold that leg level with the body.
- Clutch use in manual cars (for left hips) – may flare up symptoms in the first couple of weeks and is best avoided. Exchange cars if possible.

***Success of the Surgery***

This type of surgery is successful about 85-90% of the time. No hip operation is 100% successful in every individual. The procedure is performed to will help restore the function in your hip. The operation is most successful at relieving pain. Approximately 80% of patients my patients

say their pain is resolved, 15% say it is the same, and 5% say their pain is worse. What is harder to accomplish is the return to vigorous use of the leg in work and/or sports. Whether you can return to your previous level is an individual matter and depends on the damage to your hip, how well it heals, how well you rehabilitate and how strenuous is your desired level of work or sports. Because of the many variables involved I can make no guarantees other than to assure you I will deliver the very best medical care possible.

***Potential Risks and Complications***

All surgery carries risks, although every effort is made to minimize them. The complications can be temporary or permanent. Reassuringly, permanent complications following hip arthroscopy are rare and the majority are temporary. There are, however, risks which include the standard risks of undergoing general anaesthesia and specific risks associated with hip arthroscopy.

The complication rate after hip arthroscopy is relatively low. Complications might include, but are not limited to: infection, fracture, increased pain, bleeding, nerve palsies or injury, abandoned procedure, deep-vein thrombosis or pulmonary embolism, impotence, instrument breakage, avascular necrosis of femoral head, extravasation of irrigation fluid, delayed wound healing, exacerbation of symptoms, popping, retear of the labrum, excess bone formation, development of arthritis, stiffness, need for further surgery, and the risk of anesthesia itself. However, many of these complications are extremely rare. For example, the risk of a major complication after hip arthroscopy is substantially less than 1 in 1000.

My purpose in listing the types of complications that could possibly occur is to inform, not frighten you. While it would be preferable if we could perform surgery without any risk, this is not the case. The complications are rare (less than 1%) but regrettably, in spite of our best efforts, they do occur. I feel it is your right to know.

***Returning to Work***

For most office jobs I recommend 1-2 weeks off work. When you return to work your hip will be sore but you should be able to manage as long as you do no lifting, pushing, pulling or carrying. You will generally need 3-6 months before recovery is complete and return to heavy lifting, depending on several factors. There are no fixed rules for return to work. What I have described above are reasonable guidelines that I hope will help you and your employer ease your return to the workplace.

***Pain Medication after Surgery***

Your prescriptions including pain medication will be called into your local pharmacy. Please take your pain medication as directed. That means that you may take the pills every 4-6 hours as needed. Take the medicine with food in your stomach. Taking the medicine on an empty stomach can cause nausea. You may also notice constipation, thus a stool softener is recommended with the pain medication. When you need more medication, call our office or your pharmacy to refill the prescription. We cannot do this after 5:00 PM, as no one will be in this office. We cannot refill narcotic medication on weekends. One of my colleagues at the

Fondren Orthopedic Group is available 24 hours a day, 7 days a week but we ask that you restrict after hours and weekend call to emergencies only and let us handle less urgent problems during the week.

***Questions About Your Health***

Although the operation involves your hip, your entire body will react to the anesthesia and surgery. Your general medical condition is obviously important. Before surgery you must see the anesthesiologist. They will go over your medical condition and request any tests (blood tests or EKGs) or consultations with other physicians if necessary.

***If You Have More Questions***

We encourage you to return to the office for a further discussion at any time. We will do our best to answer any questions by phone as well. We have a lot of information on our website, www.gomberamd.com. There are surgical animations on the website as well.

***How to Schedule Surgery***

Contact us at (713) 794-3457 or info@gomberamd.com